

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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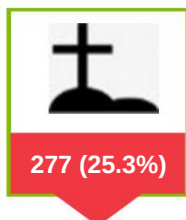
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°39/MVB_22/06/2026

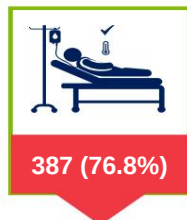
Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	• Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 22, 2026
Publication date	June 23, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



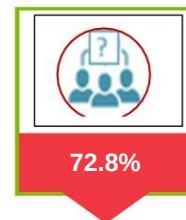
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.
further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Forty-six (46) new confirmed cases, including 10 deaths**, were reported at the end of the day on June 22, 2026, in the provinces of **Ituri (43, including 10 deaths) and North Kivu (3, including 0 deaths)**. These confirmed cases are distributed among the following health zones : **Bunia (11), Rwampara (10), Mongbwalu (8), Nizi (4), Mangala (3), Lita (3), Logo (3), and Tchomia (1)** in Ituri province, and **Beni (2) and Katwa (1)** in North Kivu province. • **Three**
- (3) patients** with Ebola virus disease in **Ituri** have been declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak.

was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

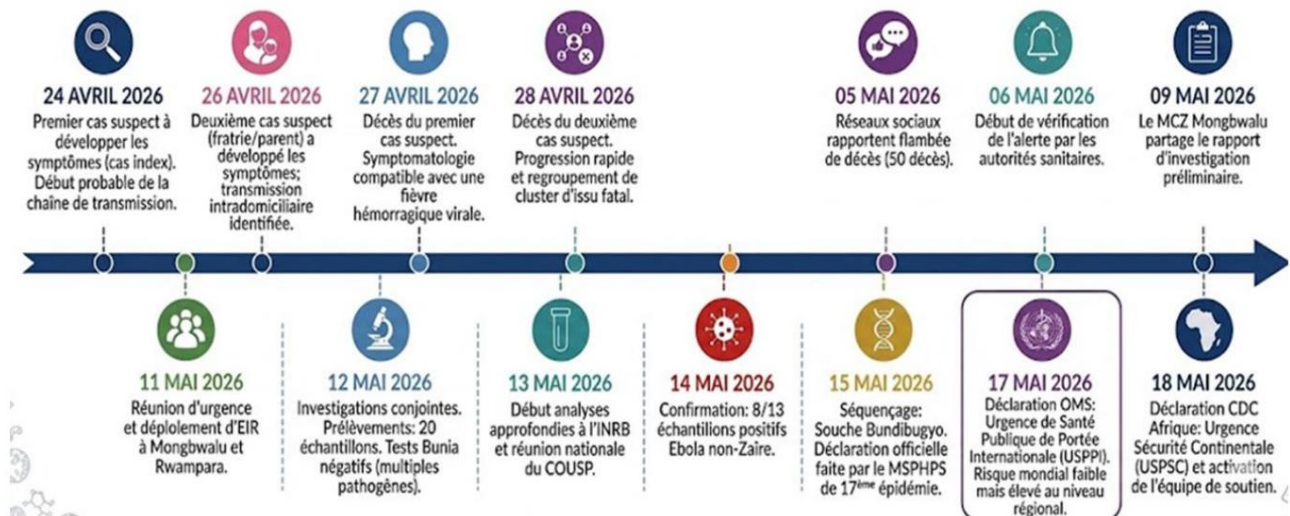


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 22, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 22, 2026.

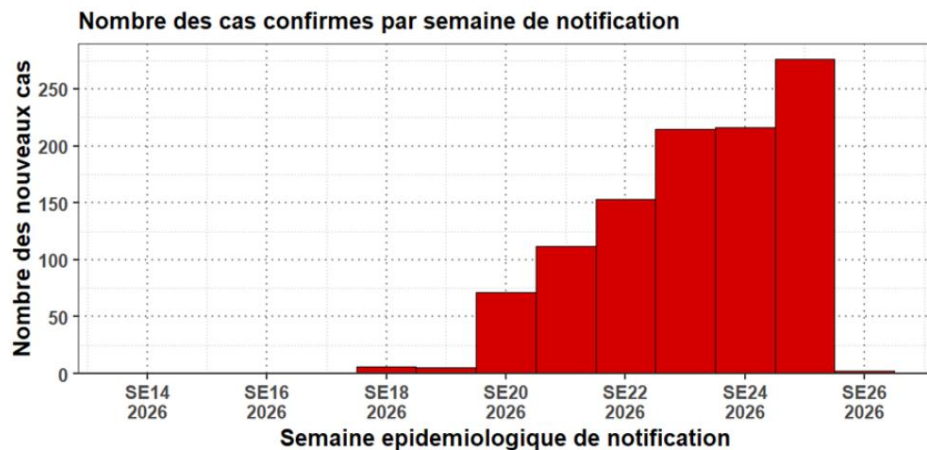
Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	997	226	22.7%	22 out of 36 (61.1%)	43
North Kivu	94	50	53.2%	11 out of 34 (32.3%)	3
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1094	277	25.3%	34 out of 104 (32.7%)	46

No new health zones were affected by the end of June 22, 2026. The number of affected health zones remains at 34/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission. However, active surveillance is ongoing. remains necessary.

3.2 Distribution by health zone (Cumulative total as of 22/06/2026)

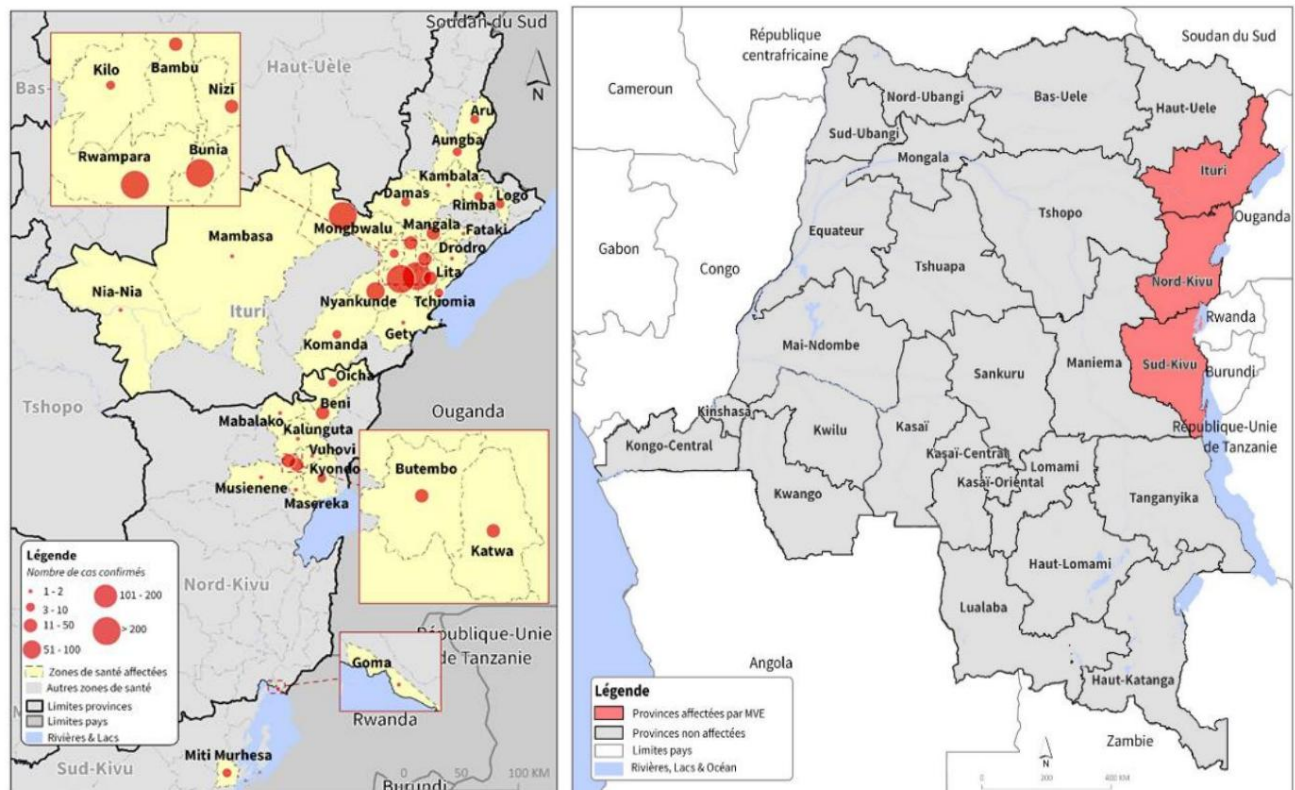
- **Ituri Province (997 cases):** Bunia (296), Rwampara (225), Mongbwalu (237), Nyankunde (86), Nizi (30), Lita (24), Mangala (14), Bambu (14), Komanda (10), Tchomia (8), Kilo (7), Aungba (6), Logo (5), Damas (4), Rimba (3), Aru (3), Mambasa (2), Nia-Nia (2), Drodoro (1), Gety (1), Kambala (1), and Fataki (1).
Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (94 cases):** Katwa (32), Butembo (28), Beni (20), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (2), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time-series analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) has contributed to early detection, confirming ongoing community transmission and increased.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 1056.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 22, 2026

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 22, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Lethality (CFR, %)
ITURI	997	226	22.7%
Bunia	296	48	16.2%
Rwampara	225	37	16.4%
Mongbwalu	237	100	42.2%
Nyankunde	86	9	10.5%
Nizi	30	6	20.0%
Lita	24	6	25.0%
Komanda	10	3	30.0%
Bamboo	14	2	14.3%
Kilo	7		14.3%
Mangala	14	1	42.9%
Tchomia	8	6	0.0%
Damascus	4	0	0.0%
Aungba	6	0	33.3%
Rimba	3	2	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	5	0	0.0%
Drodoro	1		100.0%
Gety	1	1	0.0%
Kambala	1	1 0 1 0 1	100.0%
Nia-Nia	2		100.0%
Fataki	1	20	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	94	50	53.2%
Butembo	28	13	46.4%
Katwa	32	19	59.4%
Blessed	20	12	60.0%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	3		33.3%
Goma		1	0.0%
Masereka		1	0.0%
Vuhovi			100.0%
Mabalako	1 1 1 1	0	0.0%
Musienene	2		50.0%
SOUTH KIVU	3	0 1 0 1 1	33.3%
Miti-Murhesa		1	33.3%
TOTAL	3 1094	277	25.3%

Over the past 24 hours, the epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Democratic Republic of Congo continues to worsen, with **46 new confirmed cases, including 10 deaths** have been reported, bringing the total to **1,094 confirmed cases** and **277 deaths**, expressing an overall case fatality rate of **25.3%**.

Provincial dynamics and case concentration

Approximately **91.1%** of cases (n=997) and **81.6%** of deaths (n=226), representing a case fatality rate of **22.7%**, are concentrated in **Ituri province**, making it the absolute epicenter of the epidemic. The most affected health zones are Those of Bunia (296 cases), Mongbwalu (237 cases), and Rwampara (225 cases) account for 76.0% of all reported cases in this province. In contrast, the health zones of Bunia and Rwampara have moderate case fatality rates (16.2% and 16.4% respectively), while the **Mongbwalu** zone shows marked severity.

with a case fatality rate of **42.2%** (100 deaths out of 237 cases), suggesting persistent challenges in early intervention or access to care. Furthermore, 17 additional cases were reported in as-yet-unidentified health zones, without any associated deaths, which could indicate a geographic expansion requiring further investigation.

a reinforced investigation.

Severity of the epidemic in North Kivu

A total of **94 confirmed cases and 50 deaths** have been reported in this province, representing a case fatality rate of 53.2%. This last one presents the most worrying profile in terms of severity, higher than the national average. This high mortality rate is driven by the health zones of **Beni** (60.0%), **Katwa** (59.4%) and **Oicha** (66.7%). The Katwa health zone had the highest number of reported cases (n=32), followed by the Butembo health zone (n=28) with a lower case fatality rate (46.4%). A 100% case fatality rate was exceptionally observed in the Vuhovi zone (with a small sample size), highlighting the danger posed by the virus in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Visit of the Director General of the INSP to Nyankunde accompanied by a delegation of deputies from the province of Ituri to supervise and evaluate the care of the sick: an ambulance was allocated to the health zone to address one of the gaps identified during a working session in this health zone.
- Held the coordination meeting and provided support to the response pillars. The IM also met with the data management and SitRep writing team to ensure improvements in the work within these teams.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 22, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		175	703	13	891
Alerts investigated		175	702	0	877
Investigation rate		100.0%	99.9%	0.0%	98.4%
Alerts confirmed	Alive	47	40	0	87
	Deceased	14	30	0	44
Total suspected cases for today		61	70	0	131

During the course of June 22, 2026, **891 alerts** were reported across the three provinces, **877 (98.4%)** were investigated, and **131 suspected cases, including 44 deaths**, were identified (Table 3). Not all alerts received in South Kivu were investigated due to a lack of fuel for field teams.

Table 4. Contact tracing of confirmed cases as of June 22, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	6,244	4,540	72.7%
North Kivu	1938*	1,387	71.6%
South Kivu	96	96	100.0%
Total	8,278	6023	72.8%

**The follow-up rate in North Kivu province was calculated based on the health zones that reported contact tracing data (completeness: 7/10: 70.0%)*

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- 519 new contacts** were listed, including **411** in 7 health zones in Ituri province and **108** in 5 health zones of the province of North Kivu.

Update on checkpoint and point of entry (PoC/PoE) activities

- In North Kivu** : Four (4) alerts were notified to the PoCs, including 2 in Makeke (Mabalako Health Zone), 1 in Mukulya and 1 at Mavivi airport (ZS Beni), all these alerts have been invalidated.
- In Ituri** : Two (2) alerts were notified, one at the FONER Mambasa PoC (Mambasa Health Zone) and one at the Rubingu PoC (Boga Health Zone), all were validated but the suspects refused to be isolated for sampling.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of 22 June 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	85.5% (47/55)	92.3% (12/13)	86.8% (59/68)
Number of people who went through PoE/PoC	82,504	48,419	130,923
testing, % of travelers	98.0%	97.1%	97.7%
screened, % of travelers who washed their	96.1%	97.1%	97.0%
hands, % of travelers made aware of the issue	99.1%	97.1%	98.4%
Number of alerts notified	2	4	6
Number of validated live alerts	2	0	2
Number of lifeless bodies intercepted today	0	0	0
Number of problematic contacts intercepted today	0	0	0
Percentage of validated live alerts referred to the CT/CTE	0.0%	0.0%	0.0%
% of lifeless bodies swabé	0.0%	0.0%	0.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri:** 100 samples collected and analyzed (positivity 43.0%; n=43).
- **North Kivu :** 73 new samples collected, of which 65 have been analyzed and 3 have tested positive for Ebola virus disease (positivity rate 4.6%). However, 8 samples are still being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: Completion of 14/16 safe and dignified burials (SDB), with 2 postponements; decontamination of 6 households and 2 SSHs in the Bunia health zone; start of training of 50 hygiene workers in the Bunia health zone; IPC briefings and support provided to 60 service providers in Bunia, Rwampara, Komanda and Nia-Nia; decontamination of an SSH in Mongbwalu after transfer of suspected cases; provision of IPC/WASH kits and supplies in Mambasa and Mongbwalu.

North Kivu : Decontamination of 4 healthcare facilities around confirmed cases, including 2 in Katwa (Jerusalem Hospital and Mutiri Hospital) and 2 in Beni (Gracia Hospital and Ruwenzori Hospital); establishment of two handwashing stations Hands in Malepe and at the Central Market of Beni; PCI scorecard assessment of 10 health establishments (7 in Beni, 1 in Katwa, 1 in Butembo and 1 in Kyondo): the average score is 44.3%; visit to the Kyondo Health Zone Coordination Center to assess the situation, activate the EDS team and prepare the training of supervisors and PCI focal points; provision of PCI-WASH inputs to the Kyondo Health Zone consisting of household gloves, protective glasses, face shields, chlorine, etc.; monitoring and support of 57 priority health establishments in the Health Zones of Beni, Butembo, Katwa, Kyondo, Goma and Karisimbi in order to raise the level of PCI; briefing of 202 healthcare providers from the supported health establishments on various PCI themes in the context of the Ebola epidemic.

4.5 Holistic care

- **Three (3) patients with Ebola virus disease have been declared cured** after obtaining negative control results in Ituri.
- Continued nutritional monitoring of patients in CTE/CT: **556 hot meals** were served, including 80 to suspected cases, 76 to confirmed cases, 41 to accompanying persons and 359 to PPL.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 22, 2026

Indicator of	Ituri	North Kivu	South Kivu	Together
Patients in Bed (Day -1)	298	27	10	335
Total admissions (24h)	57	36	0	93
Exits (24h)				
Deceased (Suspects/Confessions)	14	0	0	14
No cases	16	7	0	23
cured	3	0	0	3
Escapees (Suspects/Confessions)	1	0	0	1
Transferred to the HGR		0	0	0
Total outflows	0		0	41
Patients in isolation (End of Day),	34 321	7 56	10	387
including confirmed cases	173	17	0	190
(NC+AC) and suspected cases	148	39	10	197
Overall occupancy rate	87.0%	54.4%	31.3%	76.8%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 22, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (11.1%)	1 (33.3%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	68 (38.9%)	2 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	11 (17.7%)	7 (100.0%)	0
Suspected cases under follow-up that have received interventions SMSPS	50 (30.9%)	15 (100.0%)	0
Laboratory results have been announced.	26	3	1
Including positive ones	4	1	0
Number of children separated in daycare who benefited from interventions SMSPS	8 (100.0%)	0.00%	0.00%
Number of children separated in the community who have benefited from interventions SMSPS	0	1	30
Number of accompanying persons at the CTE who benefited from SMSPS interventions	29	22	4
Number of PPLs who benefited from SMSPS interventions	7	5	18
Number of household and community members who benefited from SMSPS interventions	6	28	0
Number of EDS supported	0	3	0

4.6 Continuity of care

- Monitoring of drug supplies in the ESS, with Rwampara having already distributed the drugs and Bunia/Mongbwalu having received the drugs and developed the distribution plans.

4.7 Risk communication and community engagement

Ituri: Day 2 of the CREC training workshop for multi-sectoral actors in Ituri Province continued. This was followed by a presentation on Human-Centered Design (HCD); 2,048 people were reached through educational discussions in 263 households in the Nyankunde health zone.

North Kivu: Briefing of 49 community leaders from the Makasi, Katsya and Vulindi health districts on risk communication, alert reporting and community engagement in the fight against Ebola; 9,722 people sensitized in community and health structures on Ebola prevention and rumor management in the health districts of Goma, Karisimbi, Nyiragongo and Katwa, contributing to improved community knowledge and increased acceptance of response interventions; 17 community alerts reported by 10 community actors; 12 representatives of civil society organizations (7 women and 5 men) capacity-building on Ebola, to support awareness-raising activities, early detection of alerts and community engagement in affected areas; 1 rumor collected, 1 community feedback clarified and 18 local radio stations mobilized with 3 communication formats produced and broadcast.

4.8. Logistics

- Holding the logistics pillar meeting with partners at the WFP warehouse for the organization of the monitoring of inputs and outputs.
- Delivery of IPC supplies and medicines to the health zones of Logo, Aru and Mongbwalu; • Receipt of IPC supplies and PEC by the WHO.
- Thirty-one (31) vehicles and two (2) ambulances were made available to the various pillars for operational mobility.
- Fuel supply for response vehicles.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued briefings for stakeholders, community awareness-raising on exploitation, abuse and sexual harassment, as well as the signing of codes of good conduct.
- **Ituri** : 119 people briefed (33 women and 86 men), including 57 members of the CREC in Bunia, 56 healthcare workers from the CTE CME Nyankunde in the Rwampara Health Zone and 6 WHO rental vehicle drivers; Routine activities included holding pillar meetings and coordinating with PSEA partners.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (87.0%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (72.8% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (17 cases)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (53.2%)	Deploy a laboratory catch-up plan, strengthen diagnostic capacities

9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13.	Continuity of Essential Services (CEHS) Compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

Provision of PCI WASH kits and awareness-raising in the Mongbwalu health zone at the military health post, Ituri province.



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With technical support from the World Health Organization (WHO) and
of Africa CDC